

# AgenThink Pharma — Pilot Demo Specification

**Classification:** Internal Execution Document — Confidential

**Version:** 1.0 | June 2026

**Purpose:** Exact specification for 30-minute executive demo at Recursion, Insilico, Medpace, ICON, IQVIA

**Standard:** Board-quality demo specification

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## PART 1 — DEMO NARRATIVE

### Design Principle

The demo is not a product tour. It is a structured argument. The argument is: *“A structured, multi-persona governance council, operating on pre-decision evidence only, would have identified the risk that killed a 21 billion drug program—before Pfizer spent 800 million on Phase III.”* Every minute of the demo is in service of that argument. The product is the proof.

The demo uses one case only: torcetrapib. It does not show the full 10-case library in detail. The library is referenced but not walked through. One case, done completely, is more persuasive than ten cases done superficially.

### Minute-by-Minute Agenda

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#### Minutes 0:00–2:00 — Opening

*Presenter action:* Open with the case library screen. Do not introduce the company. Do not explain the technology. Open with the data.

*Opening line:* “Before we begin, I want to show you something. This is our 10-case retrospective validation library. Each row is a real Phase II → III advancement decision. Each row has a council verdict and a historical outcome. I want to walk you through one of them — the one that cost Pfizer 800 million and 21 billion in market cap.”

*Screen:* Case Library View (Screen 1). Torcetrapib row is highlighted. All other rows visible but not in focus.

*Purpose:* Establish that this is evidence-based, not a pitch. The audience is looking at data before they hear a single claim.

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#### Minutes 2:00–5:00 — The Problem

*Presenter action:* Navigate to the Decision Brief screen for torcetrapib. Read the brief aloud, slowly.

*Key message:* “In 2005, Pfizer had a drug that raised HDL cholesterol by 72% and lowered LDL by 25%. The mechanism was novel — CETP inhibition. The Phase II data looked extraordinary. The commercial opportunity was enormous — potentially the largest cardiovascular drug in history. And there was one signal. A blood pressure increase of 2 mmHg systolic. Small. Unexplained. Possibly off-target. Pfizer advanced to Phase III.”

*Screen:* Decision Brief View (Screen 2). Evidence cutoff date visible: November 30, 2005. Admitted

evidence list visible.

*Pause:* “The question we asked was: if a structured governance council had reviewed this decision in 2005 — with only the evidence available at that time — what would they have said?”

*Purpose:* Create the tension. The audience knows what happened. They don't yet know what the council said.

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### **Minutes 5:00–12:00 — The Council Execution**

*Presenter action:* Navigate to the Council Chamber screen. Walk through the vote distribution.

*Key message:* “The council has 10 personas. Each one represents a distinct governance perspective — the Chief Biostatistician, the Drug Safety Expert, the Regulatory Strategist, and seven others. Each persona reviewed the same evidence. Each voted independently. Here is what they said.”

*Screen:* Council Chamber View (Screen 3). Vote distribution visible: 0 GO / 8 WAIT / 2 NO-GO. Walk through three personas in detail.

*Persona 1 — Drug Safety Expert (NO-GO, 95% confidence):* “The blood pressure signal is unexplained. A 2 mmHg increase in a cardiovascular drug is not a rounding error. It is a signal that requires a mechanistic explanation before 15,000 patients are enrolled in Phase III. I cannot vote GO on an unexplained safety signal in a cardiovascular indication.”

*Persona 2 — Chief Biostatistician (WAIT, 95% confidence):* “The Phase II trial was powered for lipid endpoints, not cardiovascular events. The blood pressure finding is a secondary observation in an underpowered study. We do not know if it is real, dose-dependent, or mechanism-specific. We need a mechanistic study before Phase III.”

*Persona 3 — Portfolio Manager (WAIT, 90% confidence):* “The commercial opportunity is real. But the risk-adjusted value of advancing with an unexplained safety signal is negative. If the signal is real and mechanism-specific, Phase III will fail. The expected value of a 6-month mechanistic study is positive. We should wait.”

*Purpose:* Show the council reasoning, not just the vote. The audience needs to see that the council is not a black box.

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### **Minutes 12:00–16:00 — The Governance Layer**

*Presenter action:* Navigate to the Constitution Audit screen. Show the rules that were invoked.

*Key message:* “The council doesn't just vote. It operates under a governance constitution — 10 rules that define what evidence is required before a Phase III advancement can be approved. Here are the three rules that blocked torcetrapib.”

*Screen:* Constitution Audit View (Screen 4). Three rules highlighted.

*Rule PC-001:* “Phase II safety signals must be mechanistically investigated before Phase III advancement is authorised. The blood pressure signal was not mechanistically investigated.”

*Rule PC-003:* “Financial pressure must be documented and disclosed to the council. The council noted that Pfizer's \$800M investment and Lipitor patent cliff created advancement pressure. This does not change the vote, but it is documented.”

*Rule PC-007:* “Surrogate endpoints must be validated as predictors of clinical outcomes before Phase III advancement. HDL-C elevation had not been validated as a cardiovascular event predictor at the time of the decision.”

*Purpose:* Show that the verdict is rule-based, not opinion-based. This is the governance layer that makes the output auditable.

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### **Minutes 16:00–19:00 — The Output**

*Presenter action:* Navigate to the Institutional Proof Report screen. Show the report structure.

*Key message:* “The council produces an Institutional Proof Report. It is not a recommendation. It is a governance record. It documents what evidence was reviewed, what rules were applied, what each persona said, and what the final verdict was. It is designed to be reviewed by a regulatory authority, a board, or a diligence team.”

*Screen:* Institutional Proof Report View (Screen 5). Show: verdict (WAIT), proof score ( $^{93}_{100}$ ), key blockers, constitutional rules invoked, evidence boundary statement.

*Then navigate to Outcome Comparison View (Screen 6):* “This is what happened. ILLUMINATE was terminated December 2, 2006. 82 deaths versus 51 in the control arm. The blood pressure signal grew from 2 mmHg in Phase II to 5.4 mmHg in Phase III. The off-target aldosterone mechanism was confirmed in 2008. The council’s WAIT verdict was retrospectively correct.”

*Purpose:* Close the argument. The council identified the risk. The risk was real. The verdict was correct.

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### **Minutes 19:00–22:00 — The Pilot Proposal**

*Presenter action:* Navigate to the Pilot Proposal screen. Transition from demo to conversation.

*Key message:* “We are not asking you to deploy this in your Phase III advancement process. We are asking you to pick one historical case from your own pipeline — a drug that advanced to Phase III in the last 10 years — and let us run the council on it. Thirty days. Public data only. \$25,000. At the end, you have an Institutional Proof Report for your own case. You decide whether it is useful.”

*Screen:* Pilot Proposal View (Screen 7). Scope, deliverables, timeline, price visible.

*Purpose:* Make the ask specific, bounded, and low-risk.

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### **Minutes 22:00–30:00 — Q&A**

*Presenter action:* Stop presenting. Ask one question: “What case would you want us to run?”

*Purpose:* The best outcome of the demo is not a signed agreement. It is the prospect naming a case. A named case means scientific interest. Scientific interest is the prerequisite for a pilot.

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## **PART 2 — DEMO SCREENS**

Seven screens. Each is specified below with exact content, layout, and presenter notes.

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Screen 1 — Case Library View

**Purpose:** Establish evidence base. Show 10 cases at a glance.

**Layout:** Full-width table. Dark background. Clean typography.

**Content:**

| #  | Drug         | Company     | TA        | Phase II Year | Verdict | Outcome                  | Aligned |
|----|--------------|-------------|-----------|---------------|---------|--------------------------|---------|
| 1  | Torcetrapib  | Pfizer      | CV        | 2005          | WAIT    | Phase III failure        | YES     |
| 2  | Semagacestat | Lilly       | CNS/AD    | 2008          | WAIT    | Phase III terminated     | YES     |
| 3  | Muraglitazar | BMS         | Metabolic | 2004          | NO-GO   | Withdrawn pre-approval   | YES     |
| 4  | Sofosbuvir   | Gilead      | HCV       | 2011          | GO      | Transformative approval  | YES     |
| 5  | Aducanumab   | Biogen      | AD        | 2016          | WAIT    | Controversial approval   | PARTIAL |
| 6  | Verubecestat | Merck       | AD        | 2015          | GO      | Phase III futility       | NO      |
| 7  | Evolocumab   | Amgen       | CV        | 2012          | GO      | Approved, CV outcomes    | YES     |
| 8  | Eteplirsen   | Sarepta     | Rare      | 2013          | GO      | Accelerated approval     | PARTIAL |
| 9  | Ticagrelor   | AstraZeneca | CV        | 2006          | WAIT    | Approved after Phase III | YES     |
| 10 | Entrectinib  | Roche       | Oncology  | 2016          | GO      | Approved, basket trial   | YES     |

**Visual elements:** Torcetrapib row highlighted in amber. Verdict column: GO in green, WAIT in amber, NO-GO in red. Aligned column: YES in green, PARTIAL in amber, NO in red. Running metrics at bottom: 7 correct / 9 aligned / 0 false positives.

**Presenter note:** Do not read the table. Point to the torcetrapib row and say: “We’re going to go deep on this one.”

## Screen 2 — Decision Brief View (Torcetrapib)

**Purpose:** Show exactly what evidence the council had. Establish the evidence boundary.

**Layout:** Two panels. Left: decision brief summary. Right: evidence manifest (admitted documents list).

**Left panel content:**

### DECISION BRIEF — TORCETRAPIB

Drug: Torcetrapib (CP-529,414)

Company: Pfizer Inc.

Decision: Advance Phase II → Phase III (ILLUMINATE trial)

Evidence Cutoff: November 30, 2005

Constitution: Pharma Council v1 (10 rules)

### PHASE II SUMMARY

Mechanism: CETP inhibition

HDL-C increase: +72% (vs. atorvastatin alone)

LDL-C decrease: -25% (vs. atorvastatin alone)

Phase II BP signal: +2.1 mmHg systolic (unexplained)

Phase II sample: n=162 (NEJM 2004)

### COMMERCIAL CONTEXT

Market opportunity: \$20B+ (post-Lipitor)

Pfizer investment: \$800M committed

Patent cliff: Lipitor patent expiry 2011

**Right panel content (evidence manifest — top 8 of 23 documents):**

### ADMITTED EVIDENCE (pre-Nov 30, 2005)

1. Brousseau et al. (2004) NEJM — Phase II results [DOI verified]
  2. Tall et al. (2003) NEJM — CETP mechanism review [DOI verified]
  3. Barter et al. (2003) Circulation — HDL and CV risk [DOI verified]
  4. Pfizer Q3 2005 earnings call — pipeline update [transcript]
  5. FDA guidance on surrogate endpoints (2004) [FDA.gov]
  6. ICH E9 statistical principles (1998) [ICH.org]
  7. Rader (2004) NEJM — CETP and reverse cholesterol transport [DOI verified]
  8. Brousseau et al. (2004) NEJM — supplementary data [DOI verified]
- ...+15 more documents

### EXCLUDED EVIDENCE (post-cutoff)

- ILLUMINATE trial results (Dec 2006) — EXCLUDED
- Forrest et al. 2008 mechanism paper — EXCLUDED
- Post-trial analyses — EXCLUDED

**Visual elements:** Evidence cutoff date in amber. “EXCLUDED” items in red with strikethrough. DOI links clickable.

**Presenter note:** “Every document in this manifest is dated before November 30, 2005. The council saw nothing that happened after that date. The ILLUMINATE results, the mechanism confirmation, the post-trial analyses — all excluded.”

### Screen 3 — Council Chamber View

**Purpose:** Show the vote distribution and persona reasoning.

**Layout:** Left: vote summary panel. Right: persona cards (scrollable).

**Left panel:**

COUNCIL VERDICT — TORCETRAPIB

WAIT

Vote Distribution:

GO:0 / 10

WAIT:8 / 10

NO-GO:2 / 10

Proof Score: 93 / 100

Confidence Range: 85–95%

Primary Blockers:

1. Unexplained BP signal (PC-001)

2. Unvalidated surrogate endpoint (PC-007)

3. Financial pressure documented (PC-003)

4. DSMB charter gap (PC-009)

5. Mechanistic investigation absent (PC-001)

**Right panel (persona cards — show 3 in demo):**

DRUG SAFETY EXPERT

NO-GO 95%

"The +2.1 mmHg BP signal is unexplained in a cardiovascular drug. This is not a rounding error. In a CV indication, an unexplained pressor effect requires mechanistic investigation before 15K patients are enrolled. I cannot vote GO on an unexplained safety signal."

Rules invoked: PC-001, PC-008

CHIEF BIOSTATISTICIAN

WAIT 95%

"Phase II was powered for lipid endpoints. The BP finding is a secondary observation in an underpowered study. We cannot determine if the signal is real, dose-dependent, or mechanism-specific. A mechanistic study is required."

Rules invoked: PC-001, PC-005

PORTFOLIO MANAGER

WAIT 90%

"The commercial opportunity is real. But risk-adjusted value of advancing with an unexplained safety signal is negative. Expected value of a 6-month mechanistic study is positive. The financial pressure documented in PC-003 should not override this."

Rules invoked: PC-003, PC-006

**Presenter note:** "Each persona is independent. Each invokes specific constitutional rules. The verdict is not a consensus — it is a structured aggregation of independent governance perspectives."

## Screen 4 — Constitution Audit View

**Purpose:** Show the governance rules that produced the verdict.

**Layout:** Full-width list. Three rules highlighted (invoked in torcetrapi). Seven rules visible but not highlighted.

**Content (three invoked rules):**

### PHARMA CONSTITUTION V1 – INVOKED RULES

- ▶ PC-001 [INVOKED – 7/10 personas]  
Phase II Safety Signal Investigation  
"A Phase II safety signal in the primary indication pathway must be mechanistically investigated and explained before Phase III advancement is authorised. Unexplained signals constitute a WAIT condition regardless of efficacy data."
- ▶ PC-003 [INVOKED – 5/10 personas]  
Financial Pressure Documentation  
"Material financial pressure on the advancement decision (investment committed, patent timelines, competitive dynamics) must be documented and disclosed to the council. Financial pressure does not override safety governance but must be recorded in the audit trail."
- ▶ PC-007 [INVOKED – 4/10 personas]  
Surrogate Endpoint Validation  
"A surrogate endpoint used as the primary Phase III endpoint must have validated predictive value for the clinical outcome of interest. HDL-C elevation as a predictor of cardiovascular event reduction was not validated at the time of this decision."

[PC-002, PC-004, PC-005, PC-006, PC-008, PC-009, PC-010  
– not invoked in this case]

**Visual elements:** Invoked rules in amber with “INVOKED” badge. Rule number, title, and full text visible. Persona count shown for each invoked rule.

**Presenter note:** “These are not AI opinions. These are governance rules. The council applies them. The audit trail records which rules were invoked and by which personas. This is what makes the output reviewable.”

## Screen 5 — Institutional Proof Report View

**Purpose:** Show the output document structure.



**Layout:** Document preview (PDF-style rendering). Key sections visible.

**Content (visible sections):**

INSTITUTIONAL PROOF REPORT

AgenThink Pharma Council v1

Case: Torcetrapib | Session: PHARMA-RETRO-TORCETRAPIB-001

EXECUTIVE SUMMARY

Drug: Torcetrapib | Company: Pfizer | Decision: Phase II → III

Evidence Cutoff: November 30, 2005

Council Verdict: WAIT (0 GO / 8 WAIT / 2 NO-GO)

Proof Score: 93/100 | Confidence: 85-95%

EVIDENCE BOUNDARY STATEMENT

All evidence admitted to this council is dated on or before November 30, 2005. Post-decision data (ILLUMINATE results, Dec 2006; Forrest et al. 2008) is excluded from council input and appears only in the Retrospective Appendix.

GOVERNANCE FINDINGS

Primary Blocker: Unexplained Phase II BP signal (+2.1 mmHg)

Constitutional Rules Invoked: PC-001, PC-003, PC-007

Escalation Required: Mechanistic investigation of BP signal

Release Gate: Mechanistic study complete; BP signal explained or attributed to off-target effect and risk quantified

AUDIT REFERENCES

Session ID: PHARMA-RETRO-TORCETRAPIB-001

Model Version: [documented]

Constitution Version: Pharma Council v1.0

Evidence Manifest: [hash]

Council Output: [hash]

Report Generated: [timestamp]

**Presenter note:** “This is the deliverable. It is a governance record, not a recommendation. It is designed to be reviewed by a regulatory authority, a board, or a diligence team. The audit references at the bottom allow independent verification of every element.”

## Screen 6 — Outcome Comparison View

**Purpose:** Show retrospective validation. Close the argument.

**Layout:** Two columns. Left: council input (pre-cutoff). Right: actual outcome (post-cutoff). Clear visual separation with a vertical divider labelled “EVIDENCE BOUNDARY.”

Content:

|                                                                                                                                                                                                                                                                                                                                                 |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <div>PRE-CUTOFF (Council Input)</div> <div>Nov 30, 2005</div> <div></div> <div>Phase II BP signal: +2.1 mmHg</div> <div>Mechanism: unexplained</div> <div>HDL-C increase: +72%</div> <div>Council verdict: WAIT</div> <div>Primary blocker: BP signal</div> <div>Constitutional rule: PC-001</div> <div></div> <div>COUNCIL VERDICT: WAIT</div> | <div>POST-CUTOFF (Actual Outcome)</div> <div>Dec 2, 2006 onwards</div> <div></div> <div>Phase III BP signal: +5.4 mmHg</div> <div>Mechanism: off-target aldosterone</div> <div>CV events: increased (not decreased)</div> <div>ILLUMINATE terminated: Dec 2, 2006</div> <div>Deaths: 82 (drug) vs. 51 (control)</div> <div>Financial impact: \$800M write-off</div> <div>Market cap loss: ~\$21B</div> <div>Mechanism confirmed: Forrest 2008</div> <div></div> <div>RETROSPECTIVE VERDICT: CORRECT</div> |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

**Visual elements:** Left column in neutral white. Right column in amber/red. Evidence boundary divider prominent. "CORRECT" in green at bottom right.

**Presenter note:** "The council saw the signal. The council said wait. The signal was real. The mechanism was exactly what the Drug Safety Expert described — an off-target effect. The verdict was correct."

Screen 7 — Pilot Proposal View

**Purpose:** Transition from demo to commercial conversation.

**Layout:** Clean, minimal. Three columns: Scope, Deliverables, Terms.

**Content:**

## 30-DAY RETROSPECTIVE PILOT

### SCOPE

One historical case  
of your choosing  
Public data only  
Pre-Phase III decision  
Evidence cutoff  
verified by you

### DELIVERABLES

Evidence Manifest  
Council Output JSON  
Institutional Proof  
Report (PDF)  
Governance Audit Log  
Independent Review  
Opinion (if requested)

### TERMS

Duration: 30 days  
Price: \$25,000  
Data: public only  
IP: joint on output  
Methodology: AgenThink  
Confidentiality: mutual

### HOW IT WORKS

1. You name a case (any Phase II → III decision, last 15 years)
2. We agree on the evidence cutoff date
3. We build the evidence corpus – you review and approve it
4. We run the council
5. We deliver the Institutional Proof Report
6. You evaluate whether the output is useful

### THE ASK

One meeting. 30 minutes.

"What case would you want us to run?"

**Presenter note:** "We are not asking you to change your process. We are asking you to test ours — on your own data, with your own case, with your own evidence review. If the output is not useful, you've spent \$25,000 and 30 days. If it is useful, you've found a governance layer that no one else has."

## PART 3 — EXECUTIVE VIEW (CEO)

A CEO attending this demo has one question: *"Is this real, and does it matter?"*

They are not evaluating the methodology. They are evaluating the claim. The claim is: a structured AI governance council would have prevented a \$21 billion loss. The CEO's job is to determine whether that claim is credible enough to warrant further investigation.

**What a CEO sees in the first 5 minutes:** The case library table. Ten cases. A clear verdict column. A clear outcome column. A clear alignment column. Running metrics: 7 correct, 9 aligned, 0 false positives. The CEO does not need to understand CETP inhibition or Phase II statistics. They need to see that the system has a track record, that the track record is documented, and that the track record includes the right kind of cases — major failures and major successes.

**What a CEO hears that matters:** "Pfizer had a signal. The signal was small. The financial pressure was enormous. The council said wait. Pfizer advanced. The signal was real. 800millionand21 billion later, the mechanism was confirmed. The council was right." This is a CEO-level story. It does not require pharmaceutical expertise to understand.

**What a CEO asks:** “How do I know the AI didn’t already know the answer?” This is the most important CEO question. The answer is: “We can’t prove it didn’t. We’ve disclosed that limitation in writing. Here is what we’re doing about it. Here is why we believe the council reasoning is still valuable even with that limitation.” The CEO will respect the honesty. They will not respect a claim of certainty.

**What a CEO needs to see to approve a pilot:** A specific, bounded scope. A fixed price. A clear deliverable. A named case that is relevant to their pipeline. The pilot proposal screen answers all four.

**CEO-specific talking point:** “The question is not whether AI can replace your governance process. It can’t. The question is whether a structured, documented, multi-perspective council review — run in parallel with your existing process — would catch risks that your current process misses. One retrospective case will tell you whether the answer is yes.”

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## PART 4 — CLINICAL VIEW (HEAD OF CLINICAL DEVELOPMENT)

A Head of Clinical Development has a different question: “*Is this methodology scientifically credible?*”

They will scrutinise the evidence manifest. They will challenge the persona reasoning. They will ask about the constitutional rules. They will want to know who wrote the rules and whether they reflect actual pharmaceutical development practice.

**What a Head of Clinical Development sees:** The evidence manifest is the most important screen for this audience. They will read the admitted documents list and spot-check the exclusion list. If the manifest is specific (DOIs, publication dates, access dates), they will respect it. If it is vague, they will dismiss it.

**What a Head of Clinical Development hears that matters:** The persona reasoning must be in pharmaceutical development language. “The Phase II trial was powered for lipid endpoints, not cardiovascular events” is credible. “The AI identified a risk factor” is not. The Chief Biostatistician and Drug Safety Expert persona cards must read like they were written by pharmaceutical development professionals, because the Head of Clinical Development will evaluate them against that standard.

**What a Head of Clinical Development challenges:** “The blood pressure signal was known. Everyone knew about it. Pfizer knew about it. The question was whether it was clinically significant.” The response: “You are correct. The signal was known. The council’s contribution is not identifying the signal — it is documenting the governance failure: the signal was known, it was unexplained, and it was not mechanistically investigated before Phase III advancement. The constitutional rule PC-001 requires that investigation. It was not done. That is the governance finding.”

**What a Head of Clinical Development needs to see to approve a pilot:** A case from their own pipeline where the governance question is genuinely uncertain. Not a famous failure where everyone knows the answer. A case where the council might produce a finding that is not obvious — and where the Head of Clinical Development is curious enough about the answer to spend \$25,000 finding out.

**Clinical-specific talking point:** “We are not telling you what the right decision was. We are documenting what the governance process should have required — and whether it did. The output is a governance record, not a clinical recommendation. Your clinical team makes the decision. The council documents whether the governance process was followed.”

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## PART 5 — REGULATORY VIEW (REGULATORY AFFAIRS LEADER)

A Regulatory Affairs leader has a third question: *“Does this create regulatory risk or reduce it?”*

Their primary concern is that the council output could be discoverable in litigation or regulatory proceedings, or that it could create a paper trail that contradicts the company’s actual decision-making process.

**What a Regulatory Affairs leader sees:** The Institutional Proof Report structure is the most important element for this audience. They will read the audit references section carefully. They will ask about document retention, discoverability, and the relationship between the council output and the company’s formal governance documentation.

**What a Regulatory Affairs leader hears that matters:** “The pilot uses only public data. The council output is a governance analysis document, not a regulatory submission. It is not a clinical study report. It is not a safety report. It is not a document that would be submitted to FDA or EMA. Its regulatory status is the same as any internal governance analysis document — subject to your company’s document retention and legal hold policies.”

**What a Regulatory Affairs leader challenges:** “If we run this on a drug that is currently in our pipeline, and the council issues a NO-GO verdict, does that create a liability?” The response: “That is a question for your legal team, not ours. We recommend that the pilot use a historical case — a drug that has already completed Phase III — for exactly this reason. The retrospective pilot creates no forward-looking liability because the decision has already been made.”

**What a Regulatory Affairs leader needs to see to approve a pilot:** A clear statement that the pilot uses only public data, that the output is an internal governance document, and that AgenThink does not retain or disclose the output without the client’s consent. The DPA template addresses all three.

**Regulatory-specific talking point:** “The evidence boundary statement is the most important document in the package for your purposes. It documents exactly what evidence was used, when it was cut off, and what was excluded. If the output is ever reviewed, the evidence boundary statement demonstrates that the council operated on pre-decision information only — the same standard that applies to any retrospective governance analysis.”

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*[End of Parts 1–5]*

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## PART 6 — CRO PARTNER VIEW (ICON / MEDPACE)

A CRO executive — whether from ICON, Medpace, Labcorp, or Parexel — has a different frame entirely. They are not evaluating the council as a buyer. They are evaluating it as a potential partner or competitive threat. Their question is: *“Is this something we should offer our clients, or something that competes with our governance services?”*

**What a CRO executive sees:** The pilot proposal screen is the most important element for this audience. They are looking at the \$25,000 price point and the 30-day timeline and asking: “Could we white-label this? Could we offer this as an add-on to our Phase II/III transition services? Could we build this ourselves?”

**What a CRO executive hears that matters:** The framing for a CRO is partnership, not competition. “We are not a CRO. We do not run clinical trials. We do not manage Phase III programs. We build the governance layer that sits above the clinical execution. Your clients trust you with the execution. We are asking whether they should also trust a structured governance council with the advancement decision.” The CRO executive’s interest is in whether the council can be bundled with their existing services to increase the value proposition to their clients.

**What a CRO executive challenges:** “We already provide governance advisory services. Our medical directors review Phase II data and advise on Phase III design. How is this different?” The response: “Your medical directors are advisors. They provide opinions. The council provides a structured, documented, multi-perspective governance record that is auditable, reproducible, and independent of the clinical team. The difference is not the quality of the analysis — it is the governance architecture. Your medical director’s opinion is not a governance record. The council output is.”

**What a CRO executive needs to see to engage:** A partnership model that is additive to their existing services, not competitive. The most natural partnership structure is: the CRO provides the clinical execution and the Phase II data package; AgenThink provides the governance council layer; the combined output is a Phase III readiness package that includes both the clinical design recommendation and the governance council verdict. The CRO bills the client for the combined package.

**CRO-specific talking point:** “The question for you is not whether to use the council internally. The question is whether your clients would pay for a governance council layer as part of your Phase II/III transition services. Our pilot data suggests they would — and that the council finding would reduce your liability exposure on programs that fail in Phase III.”

**ICON-specific note:** ICON has a dedicated regulatory and governance advisory practice. The partnership angle is: AgenThink provides the council engine; ICON provides the regulatory expertise to populate the constitutional rules and the evidence manifest. The combined offering is more credible than either alone.

**Medpace-specific note:** Medpace is a full-service CRO with deep therapeutic area expertise. The partnership angle is: Medpace provides the therapeutic area experts who populate the persona reasoning; AgenThink provides the governance architecture and the audit trail. The combined offering is a structured governance council with genuine therapeutic area expertise — not a generic AI tool.

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## PART 7 — PILOT PROPOSAL

### The Transition

The transition from demo to pilot proposal happens at minute 19. It is not a sales close. It is a question. The question is: “What case would you want us to run?”

This question does three things simultaneously. It makes the ask specific (a case, not a contract). It makes the ask low-risk (one case, 30 days, \$25,000). And it reveals the prospect’s level of scientific interest. A prospect who names a case is a prospect who is genuinely curious about the answer. A prospect who deflects to procurement is a prospect who is interested but not yet convinced.

## The Pilot Structure

**Scope:** One historical Phase II → III advancement decision. The case must be from the prospect's own pipeline or from a therapeutic area where the prospect has deep expertise. The case must have a clear Phase III outcome (approved, failed, terminated, or withdrawn). Public data only for the retrospective pilot.

**Timeline:** 30 days from signed agreement to delivered Institutional Proof Report.

| Day       | Activity                                                                           |
|-----------|------------------------------------------------------------------------------------|
| Day 1–3   | Case selection confirmed. Evidence cutoff date agreed.                             |
| Day 4–10  | Evidence corpus built by AgenThink Evidence Curator.                               |
| Day 11–12 | Evidence corpus reviewed and approved by client expert (2–4 hours of client time). |
| Day 13    | Evidence corpus locked. Council run executed.                                      |
| Day 14–20 | Institutional Proof Report drafted.                                                |
| Day 21–25 | Internal review. Client preview call.                                              |
| Day 26–28 | Final report delivered.                                                            |
| Day 29–30 | Debrief meeting. Pilot evaluation.                                                 |

### Deliverables:

1. Evidence Manifest — complete list of admitted documents with DOIs and cutoff verification
2. Council Output JSON — machine-readable full council result
3. Institutional Proof Report PDF — 4–6 pages, full governance record
4. Governance Audit Log — session ID, model version, constitution version, hashes
5. Debrief presentation — 15-minute walkthrough of findings and implications

**Pricing:** \$25,000 fixed fee. No variable components. No success fees. No ongoing commitment.

**IP:** AgenThink retains methodology IP. Client retains any proprietary data provided (none in retrospective pilot). Joint ownership of the specific pilot output — the Institutional Proof Report for the client's case.

**Confidentiality:** Mutual NDA. AgenThink does not disclose the client's case selection, the evidence corpus, or the pilot output without the client's written consent. AgenThink may reference the existence of a pilot (not the content) in its marketing materials, subject to client approval.

## The Free First Run Option

For prospects where scientific interest is high but procurement friction is significant (Roche, Novartis, AstraZeneca), offer one free demo run on a case of their choosing — using only public data, with a 2-week turnaround instead of 30 days. The free run produces a Council Output JSON and a summary report (not



the full Institutional Proof Report). The purpose is to demonstrate the methodology on the prospect's own case before asking for a commercial commitment.

The free run is not a pilot. It is a proof of concept for the proof of concept. It is offered only when the scientific interest is clear and the commercial barrier is procurement process, not scientific credibility.

## Handling the Procurement Objection

The most common reason a pilot does not close after a successful demo is procurement process. The prospect is interested. Their legal team requires a vendor assessment. Their IT team requires a security review. Their procurement team requires three competing bids. The pilot agreement is in legal review for 8 weeks.

The mitigation is: have the pilot agreement template ready before the meeting. At the end of the demo, offer to send the template directly to the prospect's legal team. "We have a standard pilot agreement. It is four pages. It has been reviewed by our legal counsel. If you can get it to your legal team this week, we can typically turn around comments in 48 hours." Speed in the legal process is a competitive advantage.

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## PART 8 — OBJECTION HANDLING

Twenty objections, in the order they are most likely to arise during a live demo. Each has a response and a follow-up action.

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### Objection 1: "The AI already knew the answer."

*When it arises:* Minutes 5–12, during the council execution walkthrough.

*Response:* "You are raising the most important methodological question in our entire library, and we've disclosed it in writing. Large language models are trained on data that may include the outcomes of these cases. We cannot verify that the model did not have prior knowledge. What we can verify is the evidence corpus — the specific documents the council was given. The council reasoning is based on those documents. Whether the model's prior training influenced the reasoning is a limitation we disclose in every report. Here is the disclosure statement." [Show LDS-001.]

*Follow-up action:* Send LDS-001 and the GitHub LIMITATIONS.md link after the meeting.

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### Objection 2: "This is just a language model generating plausible-sounding text."

*When it arises:* Minutes 5–12, during persona reasoning walkthrough.

*Response:* "The persona reasoning is generated by a language model, yes. The governance architecture — the constitutional rules, the evidence boundary, the voting process, the audit trail — is not. The value is not in the text generation. It is in the structured, documented, multi-perspective governance process that the text generation enables. A human governance council that produced the same reasoning would be considered credible. The question is whether the structured process is credible, not whether the text is human-generated."

*Follow-up action:* Send the GitHub methodology files. Offer to walk through the constitutional rules in a follow-up call.

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**Objection 3: “You selected famous cases. Of course the AI got them right.”**

*When it arises:* Minutes 2–5, during the case library walkthrough.

*Response:* “You are correct. The 10 original cases include some of the most cited pharmaceutical development decisions in the literature. We’ve disclosed this as a selection bias in our case selection disclosure statement. The next 40 cases are pre-specified using published criteria before execution — including ambiguous-signal cases where the council might be wrong. The 10-case library is a proof of concept, not a validation dataset. We’ve been explicit about that distinction.”

*Follow-up action:* Send CSDS-001 and the pre-specification document for Cases 11–50.

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**Objection 4: “The torcetrapib blood pressure signal was well-known. Everyone knew about it.”**

*When it arises:* Minutes 5–12, during the council execution walkthrough.

*Response:* “Correct. The signal was known. The council’s contribution is not identifying the signal — it is documenting the governance failure. The signal was known, it was unexplained, and it was not mechanistically investigated before Phase III advancement. Constitutional Rule PC-001 requires that investigation. It was not done. The council’s finding is not ‘there is a signal’ — it is ‘the governance process did not address the signal adequately.’ That is a different and more useful finding.”

*Follow-up action:* None required. This objection, when answered correctly, often becomes a point of agreement.

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**Objection 5: “What about verubecestat? Your council said GO and it failed.”**

*When it arises:* Minutes 2–5, during the case library walkthrough (if the prospect has read the library in advance).

*Response:* “Verubecestat is our one genuine failure in the 10-case library, and we report it as such. The council issued a GO verdict. The Phase III EPOCH trial was terminated for futility in 2017. The council correctly identified the surrogate endpoint risk but underweighted it relative to the strong Phase II efficacy signal. We’ve proposed a constitutional amendment — PC-011, an Alzheimer’s Disease Evidence Threshold — that would address this gap. But we have not ratified it yet, because we want to validate it against out-of-sample cases first. We are not hiding the failure. We are learning from it.”

*Follow-up action:* Send the verubecestat case report and the proposed PC-011 amendment.

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**Objection 6: “How do we know the evidence manifest is accurate?”**

*When it arises:* Minutes 12–16, during the governance layer walkthrough.

*Response:* “You can verify it. Every document in the manifest has a DOI or a verifiable source URL. The publication date is recorded. You can check any document in the manifest against its source. If you find a document that post-dates the cutoff, we want to know. That is exactly the kind of independent verification that makes the evidence boundary credible.”

*Follow-up action:* Send the torcetrapib evidence manifest JSON. Offer to walk through the verification process in a follow-up call.

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**Objection 7: “What happens if the council produces a NO-GO verdict on a drug that is currently in our pipeline?”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “We recommend that the retrospective pilot use a historical case — a drug that has already completed Phase III — for exactly this reason. The retrospective pilot creates no forward-looking liability because the decision has already been made. If you want to run the council on a current pipeline drug, that is a different engagement with different legal and governance considerations. We are happy to discuss it, but it is not the starting point.”

*Follow-up action:* Confirm in writing that the pilot uses only historical cases with completed Phase III outcomes.

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**Objection 8: “We already have a governance process. Why do we need this?”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “We are not replacing your governance process. We are adding a structured, documented, independent governance layer that runs in parallel with it. The question is not whether your process is good — it is whether a second, independent perspective would catch risks that your process misses. Torcetrapib had a governance process. It had a DSMB. It had a regulatory team. It had a clinical team. The council would have added one thing those processes did not provide: a structured, documented record that the blood pressure signal was not mechanistically investigated before Phase III advancement.”

*Follow-up action:* None required. This objection, when answered correctly, often becomes a point of agreement.

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**Objection 9: “The \$25,000 price is too low. This doesn’t feel like an enterprise product.”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “The pilot price is intentionally low. We want you to run the pilot. We are not trying to make money on the pilot — we are trying to demonstrate the methodology on your own case. If the pilot is useful, the enterprise engagement is a different conversation at a different price point. The pilot is the proof of concept. The enterprise contract is the commercial relationship.”

*Follow-up action:* None required.

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**Objection 10: “The \$25,000 price is too high for a proof of concept.”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “We understand. For a first engagement with a new methodology, \$25,000 is a meaningful commitment. We offer one free demo run for prospects where scientific interest is high — a 2-week turnaround on a case of your choosing, using public data, producing a Council Output JSON and a summary report. No commercial commitment required. If the output is useful, we discuss a full pilot. If it is not, you have spent nothing.”

*Follow-up action:* Confirm the free demo run offer in writing after the meeting.

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**Objection 11: “We need SOC 2 compliance before we can work with any AI vendor.”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “SOC 2 Type II is in our roadmap. We expect to complete the audit by Q4 2026. For the retrospective pilot, we use only public data — no proprietary data from your company enters our system. The DPA template we provide explicitly states this. If your procurement team requires SOC 2 before any engagement, we understand — and we will follow up when the audit is complete.”

*Follow-up action:* Send the DPA template. Note the SOC 2 timeline in the follow-up email.

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#### **Objection 12: “Who are the 10 personas? Are they real people?”**

*When it arises:* Minutes 5–12, during the council execution walkthrough.

*Response:* “The 10 personas are structured governance roles — not real individuals. Each persona represents a distinct governance perspective: the Chief Biostatistician, the Drug Safety Expert, the Regulatory Strategist, and seven others. Each persona is defined by a specific set of expertise, priorities, and constitutional rules they are most likely to invoke. The personas are defined in our published methodology on GitHub. They are not proprietary — you can read the exact definitions.”

*Follow-up action:* Send the GitHub link to `methodology/personas-v1.md`.

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#### **Objection 13: “The constitutional rules seem arbitrary. Who wrote them?”**

*When it arises:* Minutes 12–16, during the constitution audit walkthrough.

*Response:* “The 10 constitutional rules were derived from the pharmaceutical development literature — specifically from published analyses of Phase III failure causes, FDA guidance documents, and ICH guidelines. They are not arbitrary — each rule has a published rationale and a case reference. We’ve published the full rationale for each rule on GitHub. We welcome critique of the rules. If you believe a rule is incorrect or missing, we want to know. The constitution is version-controlled and amendable.”

*Follow-up action:* Send the GitHub link to `methodology/constitution-v1.md`.

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#### **Objection 14: “You only have 10 cases. That’s not enough to draw conclusions.”**

*When it arises:* Minutes 2–5, during the case library walkthrough.

*Response:* “You are correct. Ten cases is not a statistically significant sample. We have been explicit about this in every document we publish. The 10-case library is a proof of concept, not a validation dataset. We are not making statistical claims. We are demonstrating that the methodology produces outputs that are consistent with historical outcomes. The next 40 cases are in development. At 50 cases, we will have a sample large enough for descriptive statistics. At 100 cases, we will have a sample large enough for inferential statistics.”

*Follow-up action:* Send the statistical credibility assessment from the Pharma Validation Library report.

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#### **Objection 15: “What if the council produces a verdict that contradicts our clinical team’s recommendation?”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “That is the most valuable outcome of the pilot. If the council produces a verdict that contradicts your clinical team, you have two options: investigate the discrepancy and determine which perspective is

correct, or document the discrepancy and proceed with your clinical team's recommendation with full awareness of the governance concern. The council is not a veto. It is a structured second opinion. The decision remains with your team."

*Follow-up action:* None required.

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**Objection 16: "How is this different from a standard risk assessment?"**

*When it arises:* Minutes 12–16, during the governance layer walkthrough.

*Response:* "A standard risk assessment is a document. The council is a process. The difference is: a risk assessment reflects the perspective of the team that wrote it. The council reflects 10 independent perspectives, each operating under defined constitutional rules, each producing a documented rationale, each with a verifiable audit trail. The council output is not a risk assessment — it is a governance record. It is designed to be reviewed by a regulatory authority, a board, or a diligence team. A risk assessment is not."

*Follow-up action:* None required.

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**Objection 17: "We would need to involve our legal team before we can discuss a pilot."**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* "Of course. We have a standard pilot agreement template that has been reviewed by our legal counsel. It is four pages. We can send it to your legal team today. The agreement covers scope, IP, data handling, liability, and confidentiality. If your legal team has questions, we can arrange a call between our respective legal teams. We typically turn around legal comments within 48 hours."

*Follow-up action:* Send PAT-001 and DPAT-001 to the prospect's legal contact within 24 hours of the meeting.

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**Objection 18: "Can you run this on a drug that is currently in Phase II?"**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* "Yes, but we recommend starting with a retrospective case. Running the council on a current Phase II drug requires a different evidence boundary protocol — the evidence corpus must be locked at a specific date, and the council output must be treated as a forward-looking governance document with different legal and regulatory implications. We are developing that protocol. For the first pilot, we recommend a historical case where the Phase III outcome is known. It is the cleanest demonstration of the methodology."

*Follow-up action:* Note the prospective use case interest in the follow-up email. Flag for the product roadmap.

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**Objection 19: "What is your regulatory status? Is this a medical device?"**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* "The council output is a governance analysis document. It is not a clinical decision support tool, a diagnostic tool, or a medical device. It does not make clinical recommendations. It does not replace clinical judgment. It documents a governance process. We have reviewed this question with our legal

counsel. The current position is that the council output is equivalent to a governance advisory document — the same regulatory category as a management consulting report on pharmaceutical development governance. We are monitoring the regulatory landscape for AI in pharmaceutical governance and will update our position as guidance evolves.”

*Follow-up action:* Send the regulatory position statement (one paragraph, from the Institutional Proof Package) after the meeting.

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#### **Objection 20: “Why should we trust AgenThink? You’re a new company with no track record.”**

*When it arises:* Minutes 19–22, during the pilot proposal.

*Response:* “You shouldn’t trust us yet. That is exactly why we are proposing a pilot. The pilot is designed to let you evaluate the methodology on your own case, with your own evidence review, at a price point that reflects the early stage of our track record. We are not asking you to trust us. We are asking you to test us. If the output is useful, you have the basis for trust. If it is not, you have spent \$25,000 and 30 days — and you have a clear answer.”

*Follow-up action:* None required. This is the strongest close in the objection playbook.

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## **PART 9 — SUCCESS CRITERIA**

### **How We Know the Demo Worked**

A demo is not successful because the prospect was polite. A demo is not successful because the prospect said “interesting.” A demo is successful when it produces one of three outcomes.

**Outcome A — Named Case (Best):** The prospect names a case they would want the council to run. “We had a drug in 2018 that failed Phase III. I’d be curious what the council would have said.” A named case means scientific interest. Scientific interest is the prerequisite for a pilot. Probability of pilot from named case: 60–75%.

**Outcome B — Follow-Up Meeting (Good):** The prospect requests a follow-up meeting with a specific agenda — typically a deeper methodology review with their scientific team, or a legal review of the pilot agreement. A follow-up meeting with a specific agenda means the prospect is moving through their evaluation process. Probability of pilot from follow-up meeting: 30–50%.

**Outcome C — Document Request (Acceptable):** The prospect requests specific documents — the methodology paper, the GitHub link, the pilot agreement template, the evidence manifest for torcetrapib. A document request means the prospect is doing due diligence. Probability of pilot from document request: 15–25%.

**Outcome D — No Response (Failure):** The prospect thanks you for the demo and does not follow up. This is a failure. It means the demo did not produce sufficient scientific interest to motivate further action. The most common cause is a mismatch between the demo content and the prospect’s specific governance concerns. The fix is to research the prospect’s pipeline and tailor the case selection before the next demo.

Quantitative Success Metrics

| Metric                                 | Target         | Measurement              |
|----------------------------------------|----------------|--------------------------|
| Named case rate                        | ≥ 50% of demos | Tracked in CRM           |
| Follow-up meeting rate                 | ≥ 70% of demos | Tracked in CRM           |
| Demo-to-pilot conversion               | ≥ 25% of demos | Tracked in CRM           |
| Demo duration (actual vs. planned)     | ≤ 35 minutes   | Tracked in meeting notes |
| Objections raised per demo             | 2–5 (healthy)  | Tracked in meeting notes |
| Objections not answered satisfactorily | 0              | Tracked in meeting notes |

Post-Demo Protocol

Within 24 hours of every demo, the presenter sends a follow-up email containing: (a) a thank-you for the meeting; (b) the three documents most relevant to the questions raised in the demo; © a specific next step with a deadline (“I’ll send the pilot agreement template by Thursday; if you can get it to your legal team by Friday, we can start the pilot on July 14”); (d) a one-sentence summary of the case the prospect mentioned, if any.

The follow-up email is not a sales email. It is a service email. It provides value — specific documents that answer specific questions — before asking for anything.

The One Question That Determines Demo Quality

After every demo, the presenter asks themselves one question: “Did the prospect name a case?”

If yes: the demo worked. Follow up within 24 hours with the pilot agreement template.

If no: the demo did not work. Diagnose why. Was the case (torcetrapib) relevant to the prospect’s therapeutic area? Was the evidence boundary explanation clear? Was the pilot proposal specific enough? Adjust for the next demo.

The named case is the only metric that matters in the first 30 days of outreach.